



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمة السيد عبد الرحمن نصار

العنوان : 0

المحافظة : القاهرة

رقم البطاقة : 26311031700489 رقم التليفون : 01220023051 تاريخ تسجيل الإستمارة : 26-09-2026

العمر (تاريخ الميلاد) : 03-11-1963 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: فاطمة السيد عبد الرحمن نصار National ID: 26311031700489	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

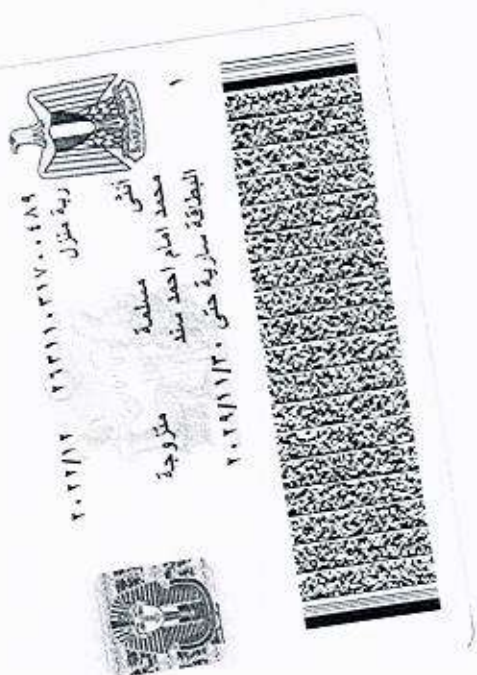
جمهورية مصر العربية
بطاقة تحقيق الشخصية

فاطمة
السيد عبد الرحمن نصار
١٤ ش ابوالمجد حجاج - عزبة بلال
الشرابية - القاهرة

٢٦٣ ١١ ٠٣ ١٧ ٠٠ ٤ ٨٩

JR7307889







Patient Name : الأستاذة / فاطمة السيد عبد الرحمن نصار	Request No. : 13001602
Patient No. : 26311031700489	Request Date : 26/10/2024
Path No. : 4092/24	Verification Date : 12/11/2024
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Bilateral breast malignant solid masses.

Nature of Specimen: Tru-cut biopsies, collected at 24/10/2024.

Gross Examination:

1- Right 2 O'clock mass:

- Four grayish white tissue cores up to 1.5 cm, totally submitted.

2- Left 6 O'clock mass:

- Four grayish white tissue cores up to 1.8 cm, totally submitted.

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA

system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

1- Right 2 O'clock mass:

* Histological type: Invasive duct carcinoma.

* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.

* Ductal carcinoma in situ: 3% (solid, cribriform pattern).

* Perineural invasion: Not detected.

* Angioinvasion: Not detected.

* Microcalcifications: Not detected.

2- Left 6 O'clock mass:

* Histological type: Invasive duct carcinoma.

* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.

* Ductal carcinoma in situ: Not detected.

* Perineural invasion: Not detected.

* Angioinvasion: Not detected.

* Microcalcifications: Not detected.

* Additional pathologic findings: Marked desmoplasia.

Verified by :

DR.Menna Nabil

أستاذة
فاطمة السيد عبد الرحمن نصار

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Patient Name : الأستاذ / فاطمة السيد عبد الرحمن نصار	Request No. : 13001602
Patient No. : 26311031700489	Request Date : 26/10/2024
Path No. : 4092/24	Verification Date : 12/11/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : ..marwa mohamed youss

Pathology Report

- Ki67: Positive in 35% of tumor cells.

For further IHC (SISH).

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Comment: Although Her2 is Negative, such cases (Score 1+) are predictive of response to Her2-Ab drug conjugates (ADCs) according to latest 2023 ASCO-CAP guidelines.

Signature

Dr. Salma Tarek.

Dr. Sagda Elkady

Verified by :

DR.Menna Nabil

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Patient Name : الأستاذ / فاطمه السيد عبد الرحمن نصار	Request No. : 13004546
Patient No. : 26311031700489	Request Date : 11/12/2024
Path No. : 4949/24	Verification Date : 31/12/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

- Free lymph nodes : (0/3)
- Left breast:
- One suspicious node out of four.
- Free margins

Microscopic examination: :

- Right breast:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1).
- Lymphovascular invasion : Not detected.
- Ductal carcinoma in situ: focal , solid and comedo.
- Lymph node involvement: Free (0/3).
- Resection Margins:
- Retroareolar disc show DCIS, comedo and solid (high grade)
- Lateral margin show DCIS.
- Free all extra resected margins.
- Additional pathologic findings: Microcalcifications.
- Left breast:
- Histological type: Invasive duct carcinoma with focal necrosis.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+2).
- Lymphovascular invasion : Not detected.
- Ductal carcinoma in situ: Not detected.
- Lymph node involvement: Positive (2/4).
- Resection Margins:
- Free nipple and areola.
- Free resected margins.

Diagnosis:

Verified by :
dr.esraa magdy

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Patient Name : الأستاذة / فاطمة السيد عبد الرحمن نصار	Request No. : 13004546
Patient No. : 26311031700489	Request Date : 11/12/2024
Path No. : 4949/24	Verification Date : 31/12/2024
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical data :

Bilateral breast masses.

Nature of specimen:

Right breast conservative surgery, right sentenil lymph node biopsy, left central quadrantectomy and left sentenil lymph node biopsy specimens collected at 11/12/2024.

Gross examination:

Six specimens were received:

1) Right breast conservative surgery: A sector of mammary tissue 8x6x2.5 cm with covering skin ellipse measured 9x2.5 cm, serial cuts revealed a firm grey white mass measured 2.5 x2x1.5 cm, located 0.5 cm from medial margin, 0.7 cm from lateral margin, 1.5 cm from skin, 2 cm from deep margin and 0.3 cm from both superior and inferior margins.

2) Right sentenil lymph node biopsy: Fibrofatty tissue piece measured 5.5x4.5 cm enclosing 3 nodules up to 1x1x0.5cm.

3) Right retro areolar disc: Fibrofatty tissue piece measured 5.5x4.5 cm.

4) Right extra margins: Three fibrofatty tissue pieces measured 13x5x2 cm labelled for lateral and superior margins, 5x3.5x1 cm labelled for medial margin and 4x2.5x0.5 cm (unlabelled).

5) Left central quadrantectomy specimen: A sector of mammary tissue 11x7x4 cm with covering skin ellipse measured 6x3 cm including nipple and areola, serial cuts revealed a firm grey white mass measured 3.5x2.5x2.5 cm, located 3.5 cm from superior margin, 1.5 cm from inferior margin and 1 cm from medial, lateral, skin and deep margins.

6) Left sentenil lymph node biopsy: A fibrofatty tissue piece measured 4 nodes measured up to 1x0.8x0.5 cm.

-Frozen section :

Right breast:

-Medial and superior margins show DCIS.

-Lateral margin is suspicious.

Verified by :

dr.esraa magdy

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Patient Name : الأستاذة / فاطمة السيد عبد الرحمن نصار	Request No. : 13004546
Patient No. : 26311031700489	Request Date : 11/12/2024
Path No. : 4949/24	Verification Date : 31/12/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Right breast mass, breast conservative surgery , sentenil lymph node biopsy specimens :

-Invasive duct carcinoma Grade II.

-Free all resected extra margins.

-Lymph nodes : Free (0/3).

-Stage : pT2, N0.

Commet: There is a disparity between frozen and paraffin results regarding right retroareolar disc. After revision of frozen results revealed free retroareolar disc which detect narrow free margin.

Left breast:

Left breast mass, central quadrantectomy and sentenil lymph node biopsy specimens:

-Invasive duct carcinoma Grade II.

- Free nipple and areola.

-Free resected margins.

-Lymph nodes : Positive (2/4).

-Stage : pT2, N1a.

Dr. Salma Tarek

Dr. Toka Medhat

Verified by :

dr.esraa magdy

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Patient Name: Fatma Al Sayed Abd El Rahman Nassar	Date : 16/11/2024
Exam : C.T CHEST	Age : 61 Year(s)
Medical ID: 3978/2024	Pat. ID: 90898

- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

- **Reported case of breast cancer revealed:**
 - Middle lobe subpleural pathy area of ground glass attenuation, likely inflammatory. For clinical correlation
 - No sizable pulmonary or hepatic lesions on non-contrast basis.

Dr. Ali Elsisy, MSc.

Prof. Menna Tallah Hanafy, M.D.



Patient Name: Fatma Al Sayed Abd El Rahman Nassar	Date : 19/10/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 61 Year(s)
Medical ID: 26311031700489	Pat. ID: 90898

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Screening

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- ACR:A

Right breast:

- UIQ irregular dense lesion covering an area of 3X3.6 cm ,about 4 cm from the nipple corresponding by US to 1-2 o'clock irregular hypoechoic lesion measuring 2.3 X1 cm about 0.3 cm from the nipple.
- Single prominent right axillary level I lymph node showing mild cortical thickening measuring 1.7X.5 cm.
- Normal skin thickness and nipple-areola complex.
- No infra or supraclavicular lymph nodes.

Left breast :

- Left deep retroareolar indistinct lesion with related linear parenchymal distortion reaching and retracting the nipple corresponding by US to irregular hypoechoic lesion measuring 3.3X2.2 cm with intraductal extension to the nipple.
- Few prominent left axillary level I lymph nodes showing cortical thickening ,largest measures 2.5X0.8 cm.
- No infra or supraclavicular lymph nodes.

Opinion:

- Bright breast UIQ and left breast deep retroareolar highly suspicious lesions with left intraductal extension (BIRADS 4c) for histopathological correlation.
- Bilateral axillary level I indeterminate lymph nodes for FNAC.

Yours truly

Signature Dr. Youmna tarek

19/10/2024 3:39 PM



Patient Name: Fatma Al Sayed Abd El Rahman Nassar	Date : 16/11/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 61 Year(s)
Medical ID: 3978/2024	Pat. ID: 90898

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:**

Liver: -

- average size (16 cm at the right MCL), displaying bright homogenous echopattern with outer surface.
- No definite focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention with normal wall thickness, no stones or diverticulae. CBD is patent and not dilated.

Spleen:

Average size measures about 11 cm with homogenous echopattern, no focal lesions.

Kidneys: -

- Both kidneys are of normal size, well differentiated & with no gross stones, or space occupying lesion could be seen.
- **Para-aortic and pancreatic area: concealed by gases**
- No ascites

Urinary Bladder: -

- Average filling with normal wall thickness with no stones or focal lesions.

- **Uterus** : is average size with homogeneous echopattern with no focal lesions.

- **Both Ovaries** : show average size with no cystic or solid lesions.

Opinion:

- Fatty liver, for laboratory correlation
- Colonic distension

Best regards
Dr. SHEREEN ALI, FEBR

Signature Dr. Sherien Ali El Sharkawy

16/11/2024 11:42 AM



Patient Name: Fatma Al Sayed Abd El Rahman Nassar	Date : 16/11/2024
Exam : C.T CHEST	Age : 61 Year(s)
Medical ID: 3978/2024	Pat. ID: 90898

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of breast cancer.

I Chest:

- Left breast parenchymal soft tissue lesion measures about 2.3 x 3 cm. subcentimetric ipsilateral axillary (level I) lymph nodes.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- Middle lobe subpleural pathy area of ground glass attenuation.
- No CT evidence of pathological mediastinal lymph node enlargement.
- Small right cardiophrenic lymph nodes.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Enlarged liver showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenal glands.

